

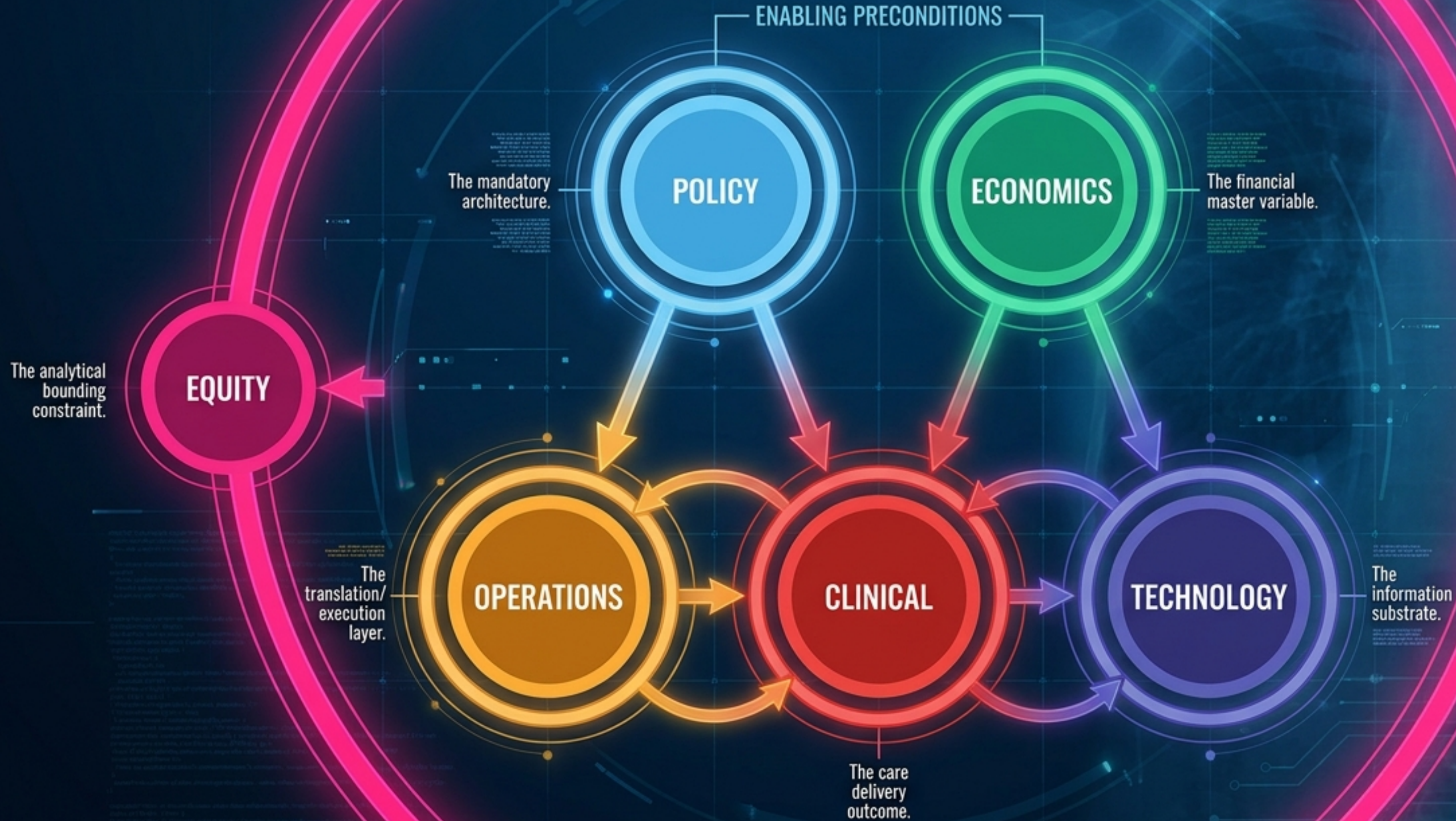
TRANSFORMING AMERICAN HEALTHCARE

A Visual Companion to the Six-Pillar Framework
and the AHS Restructuring Mandate

Incorporating Act 167/68 Legislative
Mandates, APM Financial Mechanics,
and the HTR Codebase Logic.

Prepared for: Healthcare Policymakers, Hospital
Executives, and Transformation Strategists.

CLINICAL ALGORITHM GLOBAL STYLE: THE INTERSECTING RINGS CONTROL SYSTEM

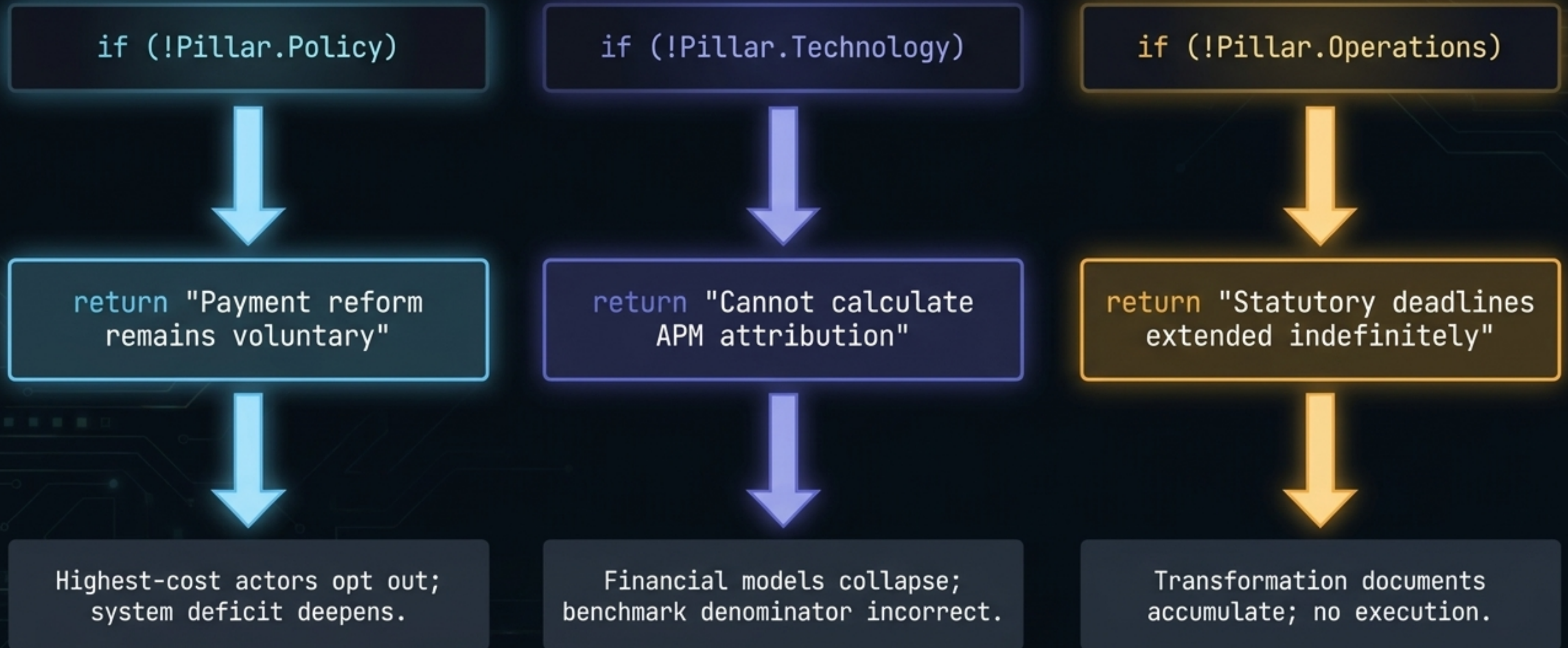


CLINICAL ALGORITHM: PILLAR INTERDEPENDENCY MATRIX

		X-AXIS: (UPSTREAM/PROVIDER)					
		 POLICY	 ECONOMICS	 OPERATIONS	 CLINICAL	 EQUITY	 TECHNOLOGY
Y-AXIS (DOWNSTREAM/RECEIVER)	 POLICY					CONSTRAINS: Equity accountability constrains every policy decision. Protects against access reduction.	
	 ECONOMICS	ENABLES: Mandatory authority enables structural payment reform. Voluntary reform fails.					REQUIRES: VBC contracts require data infrastructure. Broken data pipeline = broken benchmark.
	 OPERATIONS				REQUIRES: Care models require execution infrastructure to function beyond pilot phase.		
	 CLINICAL						
	 EQUITY						
	 TECHNOLOGY						

CLINICAL ALGORITHM GLOBAL STYLE: THE FAILURE CASCADE

OPTION B - THE LOGIC TREE



CLINICAL ALGORITHM GLOBAL STYLE: SHARED SAVINGS CALCULATOR

$\text{annualBenchmark} = \text{attributedLives} \times \text{benchmarkPMPM} \times 12$

$\text{grossSavings} = \text{annualBenchmark} - \text{actualSpend}$

$\text{actualSpend} = \text{annualBenchmark} \times (\text{actualSpendPct} / 100)$

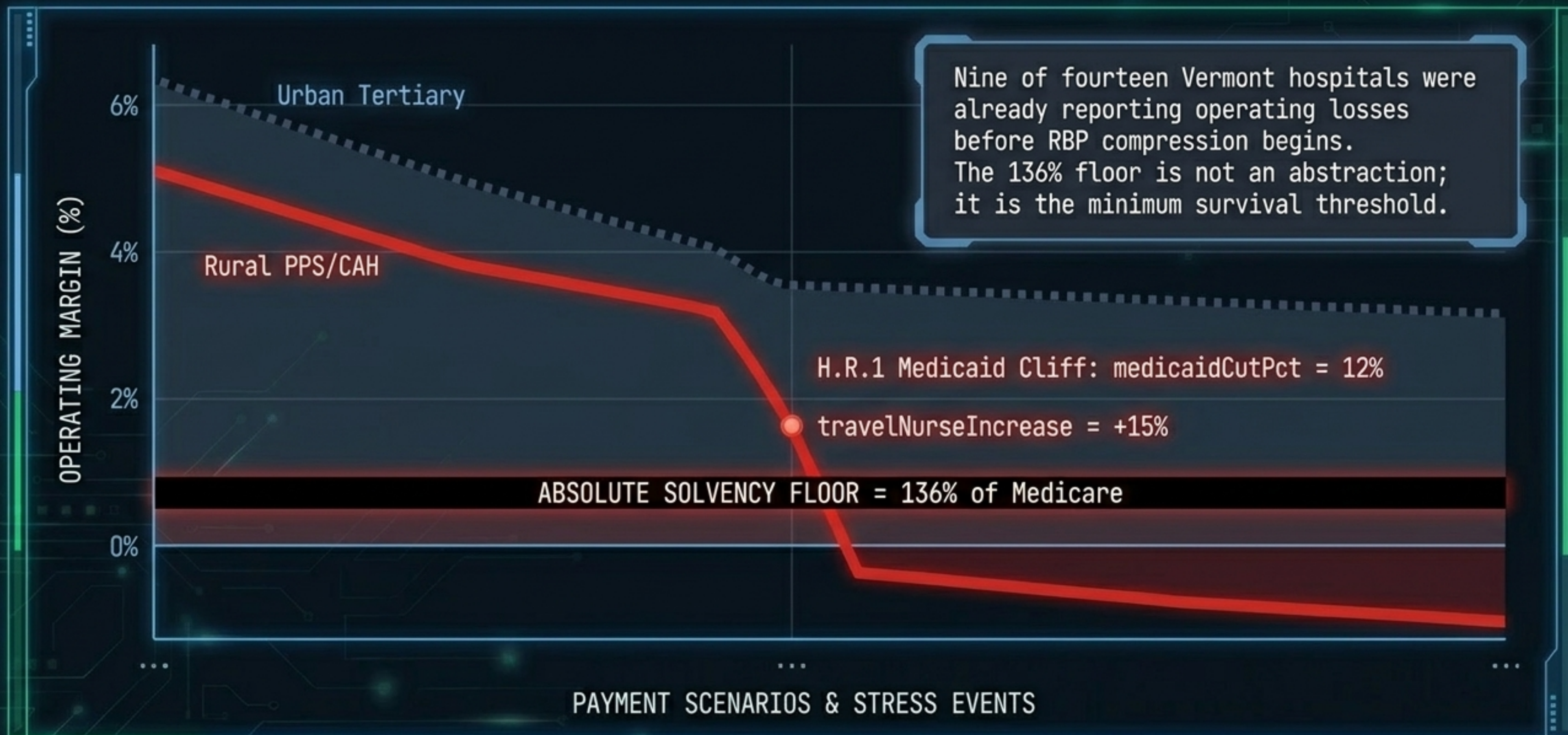
MSR Gate ($\text{savingsRate} \geq \text{model.msr}$)

lossPayment

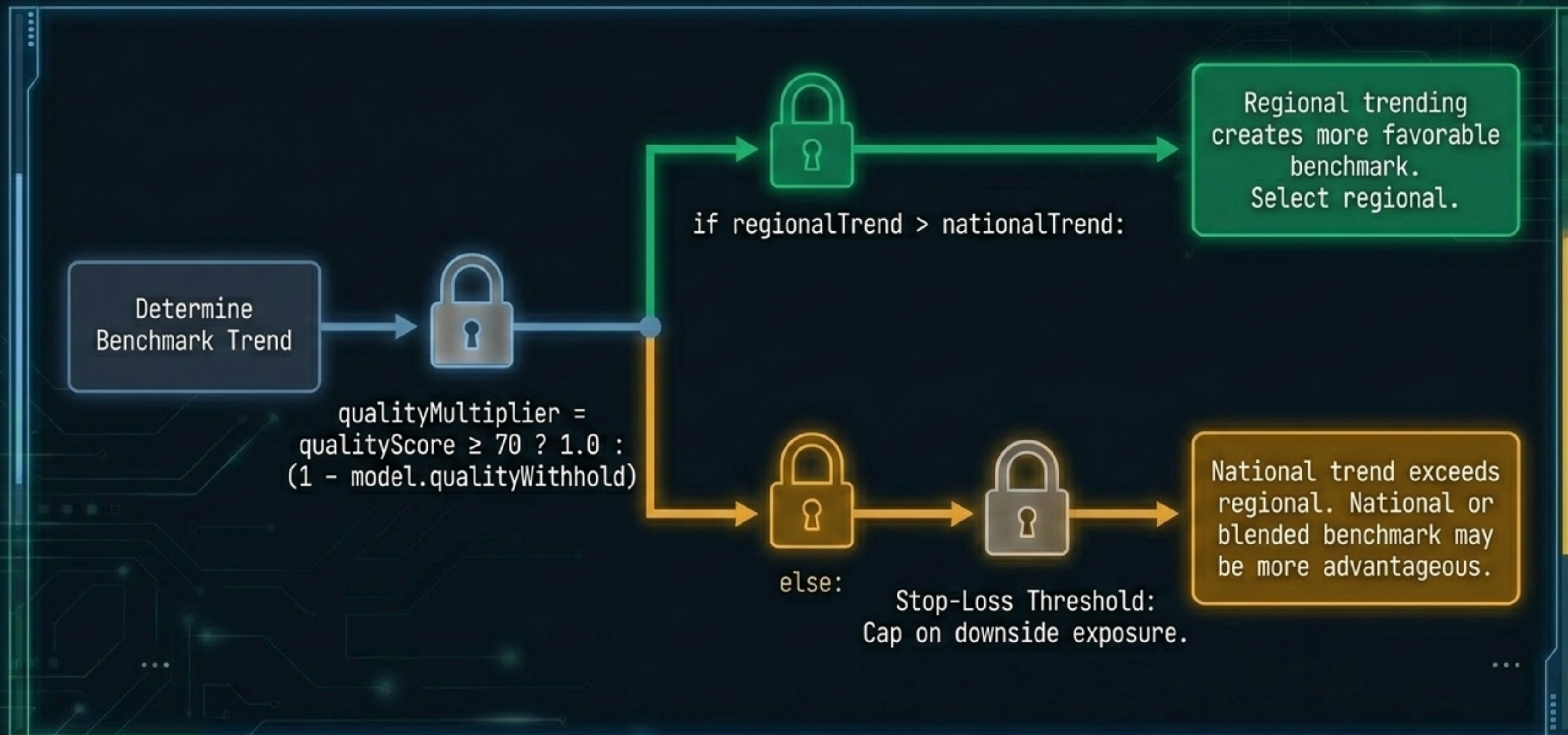
$\text{adminCost} = (\text{adminCostPMPM} \times \text{attributedLives} \times 12)$

$\text{netPosition} = \text{sharedSavings} - \text{lossPayment} - \text{adminCost}$

CLINICAL ALGORITHM GLOBAL STYLE: SOLVENCY STRESS TEST



CLINICAL ALGORITHM GLOBAL STYLE: BENCHMARK METHODOLOGY LAB



CLINICAL ALGORITHM GLOBAL STYLE: DISPARITY DECOMPOSITION

AGGREGATE STATE OUTCOME

Masks local reality

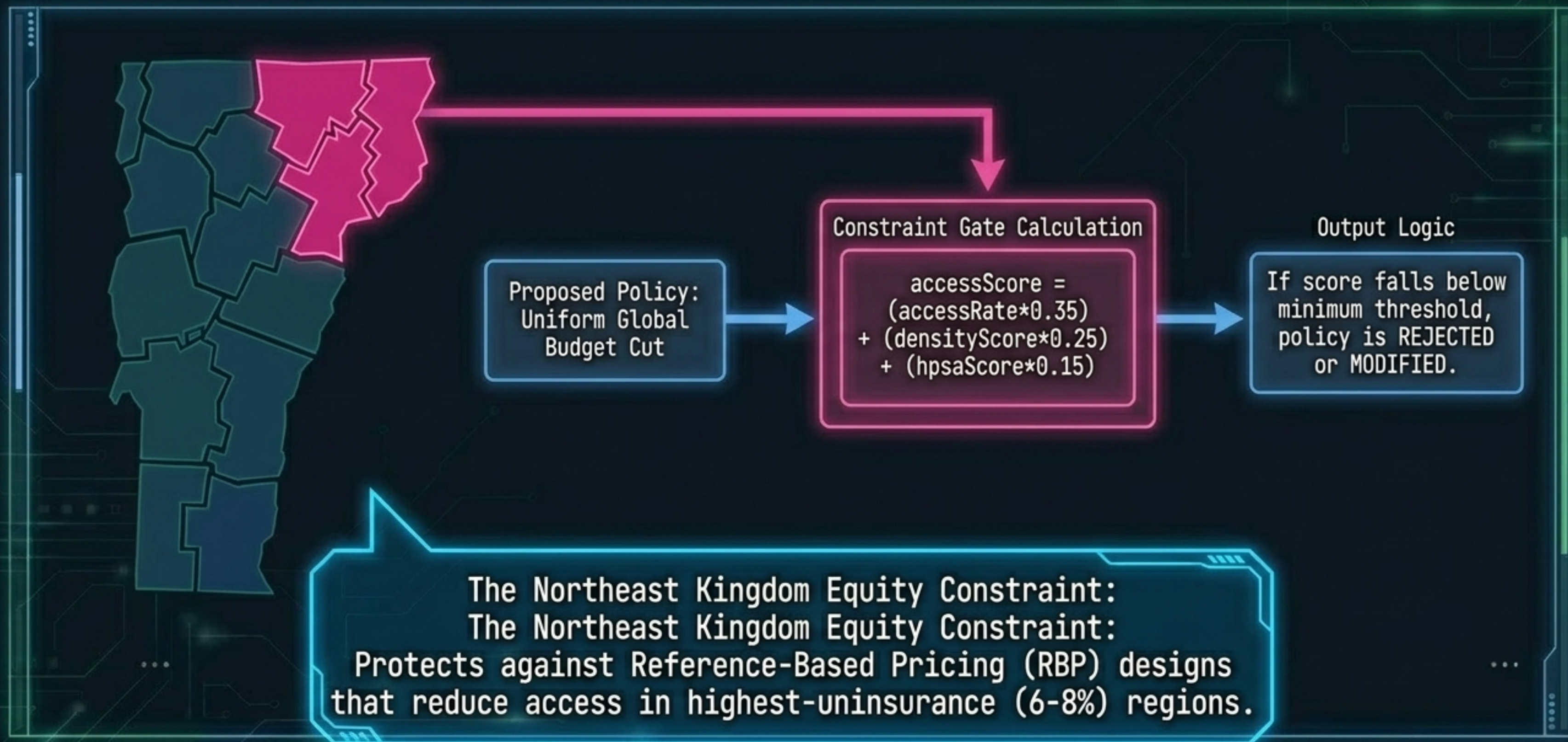
$$\text{disparityIndex} = \frac{(\text{weightedRate} - \text{allWhiteRate})}{\text{allWhiteRate}} * 100$$

Reveals the 11-point
BIPOC primary care
access gap

$$\text{costOfDisparity} = \text{excessEvents}/100K \times \text{perEventCost}$$

TRANSFORMATION THAT IMPROVES AVERAGES WHILE WIDENING
GEOGRAPHIC DISPARITIES IS A SYSTEM ARCHITECTURE FAILURE.

CLINICAL ALGORITHM GLOBAL STYLE: GEOGRAPHIC ACCESS GAP ANALYZER



CLINICAL ALGORITHM GLOBAL STYLE: 5-LAYER TARGET ARCHITECTURE - THE MONOLITHIC FOUNDATION

Application/Clinical Layer: Clinical Decision Support (CDS Hooks) & AI Governance - Point-of-care action.

Interoperability: FHIR R4 API Implementation (ONC §170.315(g)(10)) - Standardized routing.

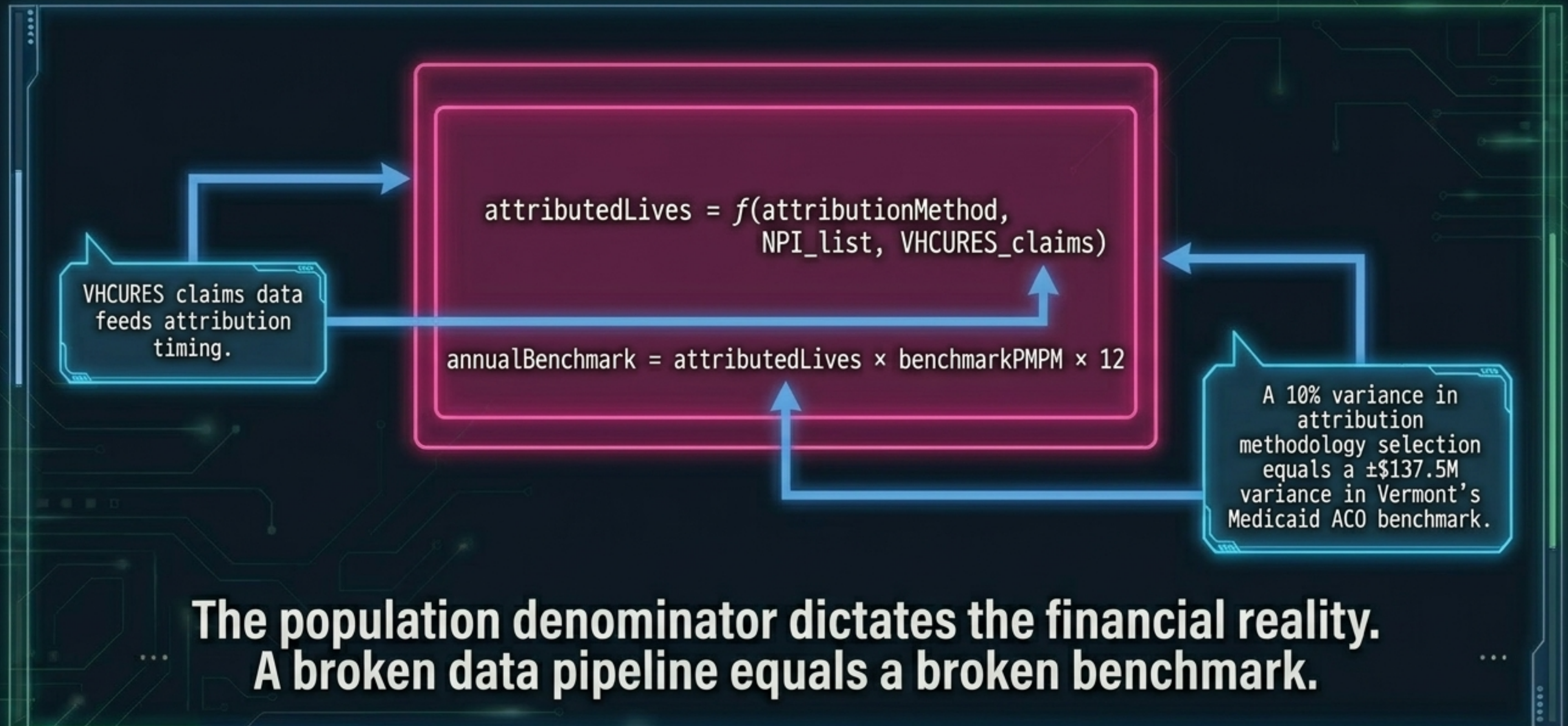
Analytics Engine: AHS/GMCB Shared Analytics Platform - The intelligence engine.

Aggregation Layer: VHCURES (All-Payer Claims Database) - Total cost of care measurement.

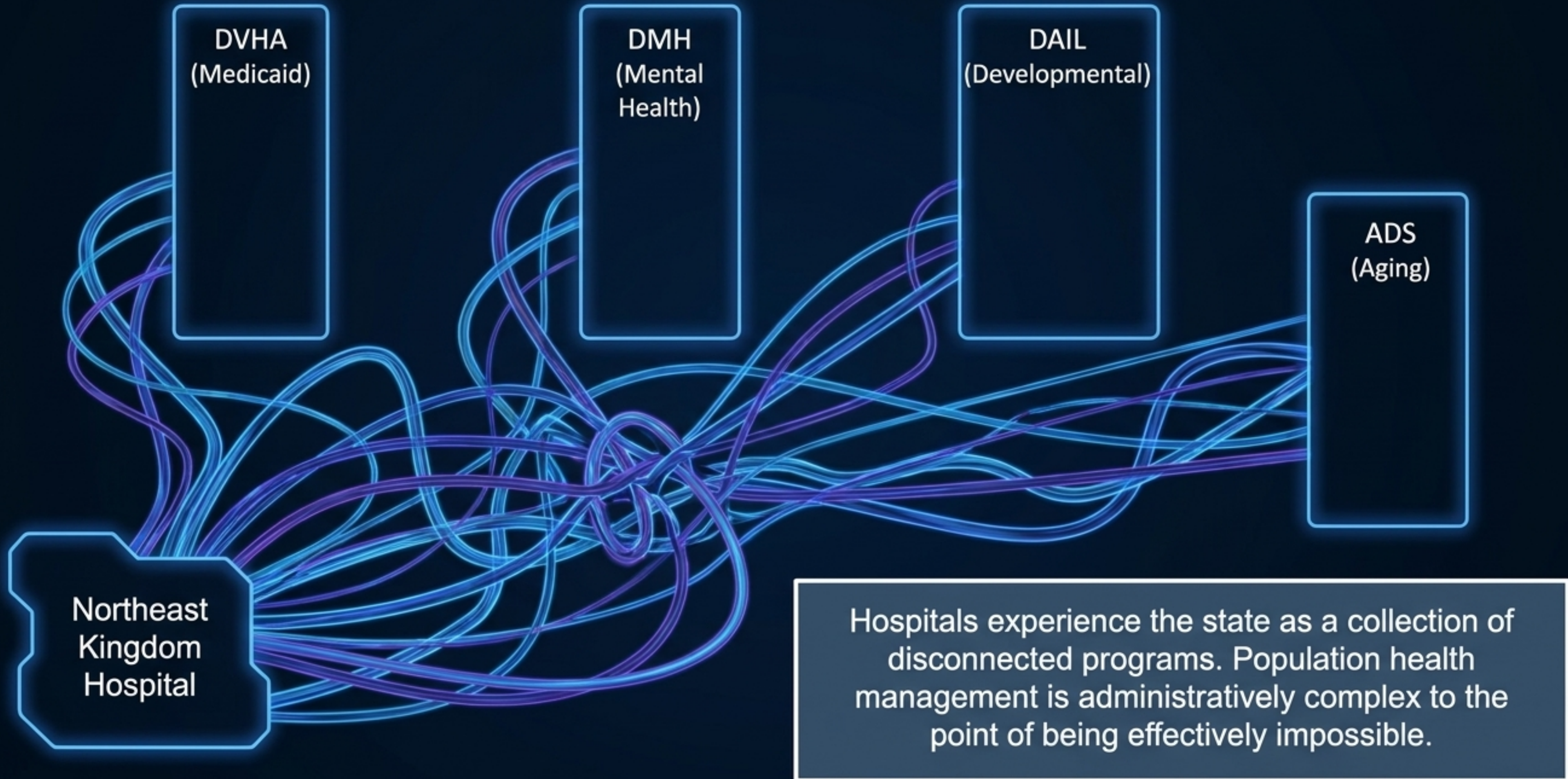
Data Ingestion: VITL / VHE (Health Information Exchange) - The raw data pipeline.

Technology is the information substrate on which every other pillar's management functions depend.

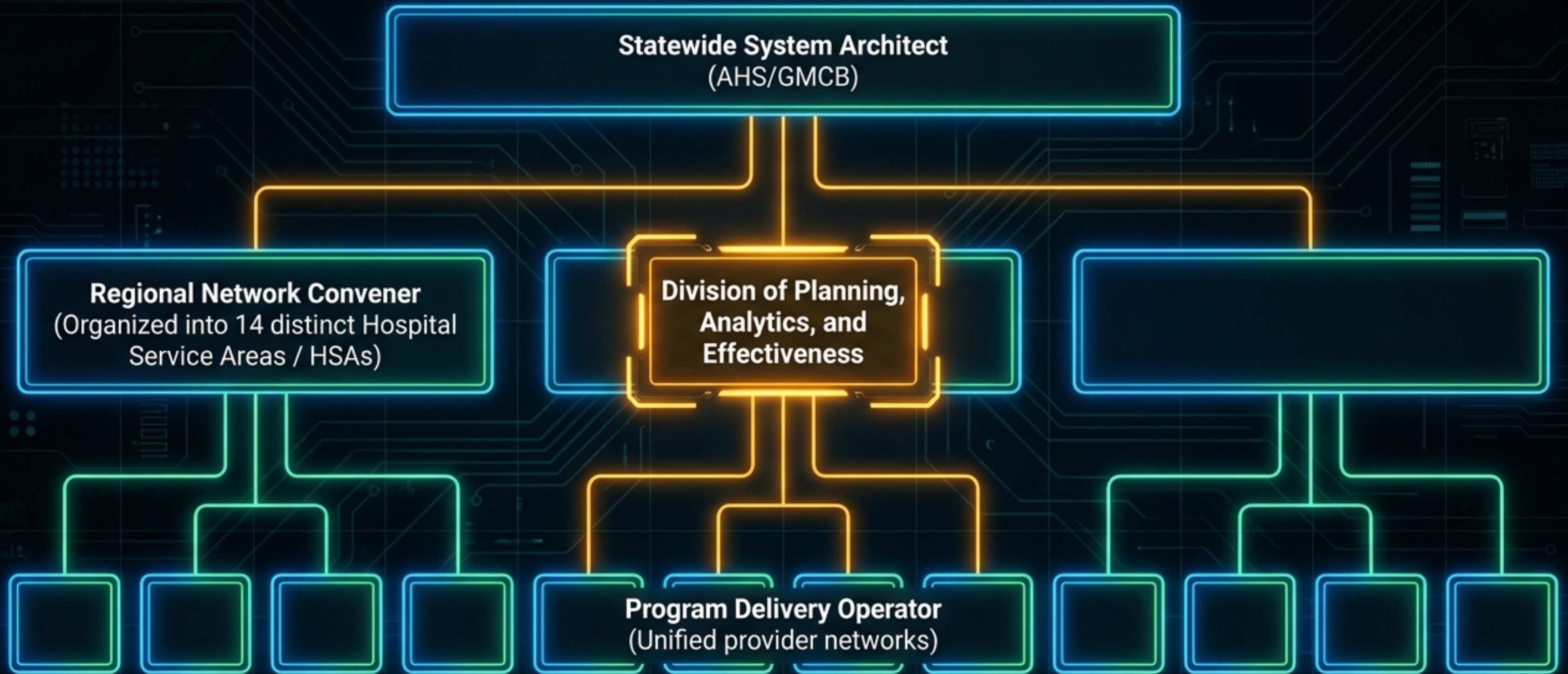
CLINICAL ALGORITHM GLOBAL STYLE: PATIENT ATTRIBUTION & FINANCIAL BENCHMARK PIPELINE



Current State: The Program Silo



Target State: Population Health System Operator



Aligning agency sub-units around geographic service areas, not funding streams.

Hub and Spoke

Healthcare Delivery

Hospitals

CCBHCs
(Mental Health)

PCMHs
(Primary Care)

Social Services / SDOH

Housing
Supports

SNAP/Food
Assistance

Non-medical
Transportation

**HSA
Coordinator**

Closing the Referral Loop.

AHS must restructure to better coordinate health and social service needs at the community level. You cannot manage global budgets without managing the social determinants that drive utilization.

1. Financial
Architecture

2. Delivery System

3. Primary/BH Care

4. Regionalization

5. Equity

6. Tech

7. Workforce

8. AHS Architecture

FY2026:
Hospital
transformation
plans submitted.

FY2027:
Mandatory RBP
(Reference-Based
Pricing) hard stop.

FY2028: Non-CAH
Global Budgets +
Dec 2028 Statewide
Delivery Plan.

FY2030:
Universal Global
Budgets for all
14 hospitals.

2026

2027

2028

2029

2030

The output must resolve the tension between the current system's \$2.4B deficit and the statutory requirement for structural transformation. The statutory timeline is non-negotiable.